Pennsylvania
Department of Human Services

Medical Assistance BULLETIN

ISSUE DATE	EFFECTIVE DATE	NUMBER
November 14, 2025	January 5, 2026	*See below

SUBJECT

Prior Authorization of Migraine Acute Treatment Agents – Pharmacy Services

BY

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IMPORTANT REMINDER: All providers must revalidate the Medical Assistance (MA) enrollment of each service location every 5 years. Providers should log into PROMISe to check the revalidation dates of each service location and submit revalidation applications at least 60 days prior to the revalidation dates. Enrollment (revalidation) applications may be found at: https://www.pa.gov/en/agencies/dhs/resources/for-providers/provider-enrollment-information/provider-enrollment-documents.html.

PURPOSE:

The purpose of this bulletin is to issue updated handbook pages that include the requirements for prior authorization and the type of information needed to evaluate the medical necessity of prescriptions for Migraine Acute Treatment Agents submitted for prior authorization.

SCOPE:

This bulletin applies to all licensed pharmacies and prescribers enrolled in the Medical Assistance (MA) Program. The guidelines to determine the medical necessity of Migraine Acute Treatment Agents will be utilized in the fee-for-service and managed care delivery systems. Providers rendering services to MA beneficiaries in the managed care delivery system should address any questions related to the prior authorization of Migraine Acute Treatment Agents to the appropriate managed care organization.

BACKGROUND:

The Department of Human Services' (Department) Pharmacy and Therapeutics (P&T) Committee reviews published peer-reviewed medical literature and recommends the following:

*01-26-28	09-26-28	27-26-28	33-26-28
02-26-28	11-26-28	30-26-28	
03-26-28	14-26-28	31-26-28	
08-26-28	24-26-28	32-26-28	

COMMENTS AND QUESTIONS REGARDING THIS BULLETIN SHOULD BE DIRECTED TO:

Fee-for-Service Provider Service Center: 1-800-537-8862

Visit the Office of Medical Assistance Programs website at:
https://www.pa.gov/en/agencies/dhs/departments-offices/omap-info.html

- Preferred or non-preferred status for new drugs and products in therapeutic classes already included on the Statewide Preferred Drug List (PDL).
- Changes to the statuses of drugs and products on the Statewide PDL from preferred to non-preferred and non-preferred to preferred.
- Therapeutic classes of drugs and products to be added to or deleted from the Statewide PDL.
- New quantity limits.
- New guidelines or revisions to existing guidelines to evaluate the medical necessity of prescriptions submitted for prior authorization.

DISCUSSION:

During the September 25, 2025, meeting, the P&T Committee recommended the following revisions to the guidelines to determine medical necessity of Migraine Acute Treatment Agents:

- Addition of guidelines for a non-preferred triptan-non-steroidal anti-inflammatory drug (NSAID) combination product that the beneficiary has a history of therapeutic failure of or a contraindication or an intolerance to the preferred triptans, has a clinical reason as documented by the prescriber why the individual active ingredients cannot be used concurrently, and also for Symbravo (meloxicam-rizatriptan), has a history of therapeutic failure of or a contraindication or an intolerance to sumatriptan-naproxen tablet.
- Addition of guidelines that for a non-preferred NSAID (e.g., Elyxyb [celecoxib] solution, diclofenac potassium powder packet) the beneficiary has a history of therapeutic failure of or a contraindication or an intolerance to the preferred oral NSAIDs (excluding ketorolac) approved or medically accepted for the beneficiary's diagnosis in the NSAIDs Statewide PDL class.

The revisions to the guidelines to determine medical necessity of prescriptions for Migraine Acute Treatment Agents submitted for prior authorization, as recommended by the P&T Committee, were subject to public review and comment and subsequently approved for implementation by the Department.

PROCEDURE:

The procedures for prescribers to request prior authorization of Migraine Acute Treatment Agents are located in SECTION I of the Prior Authorization of Pharmaceutical Services Handbook. The Department will take into account the elements specified in the clinical review guidelines (which are included in the provider handbook pages in the SECTION II chapter related to Migraine Acute Treatment Agents) when reviewing the prior authorization request to determine medical necessity.

As set forth in 55 Pa. Code § 1101.67(a), the procedures described in the handbook pages must be followed to ensure appropriate and timely processing of prior authorization requests for drugs and products that require prior authorization.

ATTACHMENTS:

Prior Authorization of Pharmaceutical Services Handbook - Updated pages

RESOURCES:

Prior Authorization of Pharmaceutical Services Handbook – SECTION I
Pharmacy Prior Authorization General Requirements
https://www.pa.gov/en/agencies/dhs/resources/pharmacy-services/pharmacy-prior-authorization-general-requirements.html

Prior Authorization of Pharmaceutical Services Handbook – SECTION II
Pharmacy Prior Authorization Guidelines
https://www.pa.gov/en/agencies/dhs/resources/pharmacy-services/clinical-guidelines.html

I. Requirements for Prior Authorization of Migraine Acute Treatment Agents

A. Prescriptions That Require Prior Authorization

Prescriptions for Migraine Acute Treatment Agents that meet any of the following conditions must be prior authorized:

1. A non-preferred Migraine Acute Treatment Agent. See the Preferred Drug List (PDL) for the list of preferred Migraine Acute Treatment Agents at: https://papdl.com/preferred-drug-list.

2. A Migraine Acute Treatment Agent with a prescribed quantity that exceeds the quantity limit. The list of drugs that are subject to quantity limits, with accompanying quantity limits, is available at: https://www.pa.gov/en/agencies/dhs/resources/pharmacy-services/quantity-limits-daily-dose-limits.html.

3. A Migraine Acute Treatment Agent when there is a record of a recent paid claim for another Migraine Acute Treatment Agent in the point-of-sale on-line claims adjudication system (therapeutic duplication).

4. A prescription for a small molecule calcitonin gene-related peptide (CGRP) receptor antagonist (gepant).

5. A prescription for a serotonin (5-HT) 1F receptor agonist (ditan).

6. A prescription for an ergot alkaloid.

B. Review of Documentation for Medical Necessity

In evaluating a request for prior authorization of a prescription for a Migraine Acute Treatment Agent, the determination of whether the requested prescription is medically necessary will take into account the whether the beneficiary:

1. For a gepant for the preventive treatment of migraine, see the prior authorization guideline related to Migraine Prevention Agents; **OR**

2. **Both** of the following:

   a. Is being treated for a diagnosis that is indicated in the U.S. Food and Drug Administration (FDA)-approved package labeling OR a medically accepted indication
   b. Has a diagnosis confirmed according to the current International Headache Society Classification of Headache Disorders;

   AND

3. Is age-appropriate according to FDA-approved package labeling, nationally recognized

compendia, or peer-reviewed medical literature; **AND**

4. Is prescribed a dose that is consistent with FDA-approved package labeling, nationally recognized compendia, or peer-reviewed medical literature; **AND**

5. Does not have a contraindication to the prescribed drug; **AND**

6. For a gepant for the acute treatment of migraine, **one** of the following:

   a. Has a history of therapeutic failure of at least two (5-HT $_{1B/1D}$) receptor agonists (triptans)
   b. Has a contraindication or an intolerance to the preferred triptans;

   AND

7. For a ditan, has a history of therapeutic failure of or a contraindication or an intolerance to the preferred triptans; **AND**

8. For an ergot alkaloid~~s~~, has a history of therapeutic failure of or a contraindication or an intolerance to standard first-line abortive drugs based on headache classification as recommended by current consensus guidelines (such as guidelines from the American Academy of Neurology, American Academy of Family Physicians, American Headache Society); **AND**

9. For a non-preferred Migraine Acute Treatment Agent, **one** of the following:

   a. For a non-preferred triptan, has a history of therapeutic failure of or a contraindication or an intolerance to the preferred triptans,

   b. For a non-preferred gepant, has a history of therapeutic failure of or a contraindication or an intolerance to the preferred gepants,

   c. For a non-preferred non-steroidal anti-inflammatory drug (NSAID) (e.g., Elyxyb [celecoxib] solution, diclofenac potassium powder packet), has a history of therapeutic failure of or a contraindication or an intolerance to the preferred oral NSAIDs (excluding ketorolac) approved or medically accepted for the beneficiary's diagnosis in the NSAIDs Statewide PDL class,

   d. For a non-preferred triptan-NSAID combination product, **all** of the following:

      i. Has a history of therapeutic failure of or a contraindication or an intolerance to the preferred triptans,
      ii. Has a clinical reason as documented by the prescriber why the individual active ingredients cannot be used concurrently,
      iii. In addition, for Symbravo (meloxicam-rizatriptan), has a history of therapeutic failure

of or a contraindication or an intolerance to sumatriptan-naproxen tablet,

e. For non-preferred Migraine Acute Treatment Agents other than triptans, gepants, NSAIDs, and triptan-NSAID combination products (e.g., ditans, ergot alkaloids, etc.), has a history of therapeutic failure of or a contraindication or an intolerance to the preferred Migraine Acute Treatment Agents approved or medically accepted for the beneficiary's diagnosis or indication;

AND

10. For therapeutic duplication, **one** of the following:

    a. Is being titrated to or tapered from another drug in the same class
    b. Has a medical reason for concomitant use of the requested drugs that is supported by peer-reviewed medical literature or national treatment guidelines;

AND

11. If a prescription for a Migraine Acute Treatment Agent is for a quantity that exceeds the quantity limit, the determination of whether the prescription is medically necessary will also take into account **all** of the following:

    a. The guidelines set forth in the Quantity Limits Chapter,

    b. Whether the beneficiary is prescribed the requested drug by **one** of the following:

        i. A neurologist
        ii. A headache specialist who is certified in headache medicine by the United Council for Neurologic Subspecialties (UCNS),

    c. For the acute treatment of migraine, **both** of the following:

        i. **One** of the following:

            a) The beneficiary is using the requested drug in addition to at least one drug for migraine prevention (e.g., beta-blocker, anticonvulsant, antidepressant, CGRP monoclonal antibody, gepant, botulinum toxin)
            b) The beneficiary has a history of therapeutic failure of or a contraindication or an intolerance to all preventive migraine drugs recommended by current consensus guidelines (such as guidelines from the American Academy of Neurology, American Academy of Family Physicians, American Headache Society)

        ii. Has documentation of an evaluation for the overuse of abortive drugs, including opioids.

NOTE: If the beneficiary does not meet the clinical review guidelines listed above but, in the professional judgment of the physician reviewer, the services are medically necessary to meet the medical needs of the beneficiary, the request for prior authorization will be approved.

FOR RENEWALS OF PRIOR AUTHORIZATION FOR A MIGRAINE ACUTE TREAMENT AGENT: The determination of medical necessity of a request for renewal of a prior authorization for a Migraine Acute Treatment Agent that was previously approved will take into account whether the beneficiary:

1. Is prescribed a dose that is consistent with FDA-approved package labeling, nationally recognized compendia, or peer-reviewed medical literature; **AND**

2. Does not have a contraindication to the prescribed drug; **AND**

3. Has documentation of improvement in headache pain, symptoms, or duration; **AND**

4. For a non-preferred Migraine Acute Treatment Agent, **one** of the following:

    a. For a non-preferred triptan, has a history of therapeutic failure of or a contraindication or an intolerance to the preferred triptans,

    b. For a non-preferred gepant, has a history of therapeutic failure of or a contraindication or an intolerance to the preferred gepants,

    c. For a non-preferred NSAID (e.g., Elyxyb [celecoxib] solution, diclofenac potassium powder packet), has a history of therapeutic failure of or a contraindication or an intolerance to the preferred oral NSAIDs (excluding ketorolac) approved or medically accepted for the beneficiary's diagnosis in the NSAIDs Statewide PDL class,

    d. For a non-preferred triptan-NSAID combination product, **all** of the following:

        i. Has a history of therapeutic failure of or a contraindication or an intolerance to the preferred triptans,
        ii. Has a clinical reason as documented by the prescriber why the individual active ingredients cannot be used concurrently,
        iii. In addition, for Symbravo (meloxicam-rizatriptan), has a history of therapeutic failure of or a contraindication or an intolerance to sumatriptan-naproxen tablet,

    e. For non-preferred Migraine Acute Treatment Agents other than triptans, gepants, NSAIDs, and triptan-NSAID combination products (e.g., ditans, ergot alkaloids, etc.), has a history of therapeutic failure of or a contraindication or an intolerance to the preferred Migraine Acute Treatment Agents approved or medically accepted for the beneficiary's diagnosis or indication;

AND

5. For therapeutic duplication, **one** of the following:

   a. Is being titrated to or tapered from another drug in the same class
   b. Has a medical reason for concomitant use of the requested drugs that is supported by peer-reviewed medical literature or national treatment guidelines;

   AND

6. If a prescription for a Migraine Acute Treatment Agent is for a quantity that exceeds the quantity limit, the determination of whether the prescription is medically necessary will also take into account **all** of the following:

   a. The guidelines set forth in the Quantity Limits Chapter,

   b. Whether the beneficiary is prescribed the requested drug by **one** of the following:

      i. A neurologist
      ii. A headache specialist who is certified in headache medicine by the UCNS,

   c. For the acute treatment of migraine, **both** of the following:

      i. **One** of the following:

         a) The beneficiary is using the requested drug in addition to at least one drug for migraine prevention (e.g., beta-blocker, anticonvulsant, antidepressant, CGRP monoclonal antibody, gepant, botulinum toxin)
         b) The beneficiary has a history of therapeutic failure of or a contraindication or an intolerance to all preventive migraine drugs recommended by current consensus guidelines (such as guidelines from the American Academy of Neurology, American Academy of Family Physicians, American Headache Society)

      ii. Has documentation of an evaluation for the overuse of abortive drugs, including opioids.

   NOTE: If the beneficiary does not meet the clinical review guidelines listed above but, in the professional judgment of the physician reviewer, the services are medically necessary to meet the medical needs of the beneficiary, the request for prior authorization will be approved.

C. Clinical Review Process

Prior authorization personnel will review the request for prior authorization and apply the clinical guidelines in Section B. above to assess the medical necessity of a prescription for a

Migraine Acute Treatment Agent. If the guidelines in Section B. are met, the reviewer will prior authorize the prescription. If the guidelines are not met the prior authorization request will be referred to a physician reviewer for a medical necessity determination. Such a request for prior authorization will be approved when, in the professional judgment of the physician reviewer, the services are medically necessary to meet the medical needs of the beneficiary.

D. References

1. (2021), 63rd Annual Scientific Meeting American Headache Society®. Headache: The Journal of Head and Face Pain, 61: 1-178. https://doi.org/10.1111/head.14130
2. Almotriptan Package Insert. North Wales, PA: Teva Pharmaceuticals USA, Inc.; May 2017.
3. American Headache Society. The American Headache Society position statement on integrating the new migraine treatments into clinical practice. Headache. 2019;59:1-18.
4. D.H.E. 45 Package Insert. Bridgewater, NJ: Bausch Health US, LLC; April 2022.
5. Diclofenac potassium powder Package Insert. Fairfield, NJ: Leading Pharma, LLC; January 2024.
6. Elyxyb Package Insert. Raleigh, NC: BioDelivery Sciences International, Inc.; September 2021.
7. Frova Package Insert. Malvern, PA: Endo Pharmaceuticals Inc.; August 2018.
8. Imitrex Package Insert. Durham, NC: GlaxoSmith-Kline; February 2024.
9. Institute for Clinical Systems Improvement. Diagnosis and treatment of headache. eleventh edition. January 2013.
10. International Headache Society. Headache Classification Committee of the International Headache Society (IHS): the international classification of headache disorders, 3rd edition. Cephalalgia. 2018; Vol. 38(1):1-211.
11. Mack KJ. Acute treatment of migraine in children. Patterson MC, Swanson JW, Goddeau Jr. RP, eds. Waltham, MA: UpToDate Inc. Updated May 08, 2023. Accessed August 19, 2024.
12. Maxalt Package Insert. Jersey City, NJ: Organon LLC; June 2021.
13. Mayans L, Walling A. Acute migraine headache: treatment strategies. American Family Physician. 2018;97(4):243-251.
14. Migranal Package Insert. Bridgewater, NJ: Bausch Health US, LLC; September 2022.
15. Naratriptan Package Insert. Princeton, NJ: Bionpharma Inc.; December 2023.
16. Nurtec ODT Package Insert. New York, NY: Pfizer Labs; August 2025.
17. Relpax Package Insert. New York, NY: Roerig Division of Pfizer Inc.; March 2020.
18. Reyvow Package Insert. Indianapolis, IN: Eli Lilly and Company; September 2022.
19. Schwedt TJ, Garza I. Acute treatment of migraine in adults. Swanson JW, Goddeau Jr. RP, eds. Waltham, MA: UpToDate Inc. Updated July 22, 2024. Accessed August 19, 2024.
20. Symbravo Package Insert. New York, NY: Axsome Therapeutics, Inc.; February 2025.
21. Tosymra Package Insert. Princeton, NJ: Promius Pharma, LLC; January 2019.
22. Treximet Package Insert. Brentwood, TN: Currax Pharmaceuticals LLC; November 2024.
23. Trudhesa Package Insert. Milano, Italy: Mipharm, S.p.A.; August 2023.
24. Ubrelvy Package Insert. North Chicago, IL: AbbVie Inc.; June 2025.
25. Zavzpret Package Insert. New York, NY: Pfizer Labs; August 2025.
26. Zembrace SymTouch Package Insert. Princeton, NJ: Promius Pharma; June 2019.

27. Zomig Package Insert. Bridgewater, NJ: Amneal Specialty; March 2022.